Northbridge General Hospital (fictional)
100 Example Avenue, Northbridge - fictional address, telephone 000 0000

Radiology Report - CT Chest, Abdomen and Pelvis

Department of Radiology

PATIENT	UHID	AGE / SEX
Thomas Whitfield	**MG25006**	**49 years / Male**
REPORT DATE	REFERENCE NO.	DEPARTMENT
23 Sep 2026	**IMG-67409931**	**Department of Radiology**

EXAMINATION	INDICATION	COMPARISON
CT chest, abdomen and pelvis with intravenous contrast	**Stage III sigmoid cancer, end of adjuvant chemotherapy**	**CT 02 Feb 2026 (pre-operative)**

Technique

Helical acquisition from the lung apices to the pubic symphysis in the portal venous phase after 100 mL of intravenous iodinated contrast. Oral contrast was not given.

Comparison of Measurable Findings

Finding	CT 02 Feb 2026	CT 23 Sep 2026	Change
Right lower lobe subpleural nodule	4 mm	4 mm	Unchanged
Liver segment 6 hypoattenuating focus	4 mm	4 mm	Unchanged
Largest retroperitoneal node (short axis)	6 mm	5 mm	Smaller
Sigmoid anastomosis wall thickness	Not applicable	4 mm	Post-surgical

Findings: Chest

The lungs are clear. A 4 mm subpleural nodule in the right lower lobe is unchanged since February 2026 and is considered benign. No pleural or pericardial effusion. No enlarged mediastinal or hilar lymph nodes.

Findings: Abdomen

No liver metastases. A 4 mm hypoattenuating focus in segment 6, too small to characterise and unchanged, most likely a simple cyst. The spleen, pancreas, adrenal glands and kidneys are normal. No ascites. No retroperitoneal lymphadenopathy.

Findings: Pelvis

Post-surgical changes at the sigmoid colon anastomosis with no thickening or mass. No pelvic lymphadenopathy or free fluid. The bladder and prostate are unremarkable.

Findings: Skeleton

No suspicious osseous lesion.

Findings: Lymph Nodes and Vessels

No enlarged axillary, mediastinal, mesenteric, retroperitoneal or pelvic lymph nodes by size criteria. The largest retroperitoneal node measures 5 mm in short axis. The portal and mesenteric veins are patent; the aorta is of normal calibre.

Recommendation

No additional imaging is needed at this stage. Continue the surveillance schedule agreed with the oncology team and compare any future examination with this study.

SYNTHETIC DATA — FOR DEMONSTRATION ONLY

Radiation Dose

Dose-length product 1,150 mGy.cm for the combined examination. Dose modulation was applied.

Impression

- No CT evidence of recurrent or metastatic disease.
- Stable 4 mm right lower lobe nodule; no further imaging required for it.
- Routine surveillance as per oncology protocol.

Electronically signed: **Dr. Nikhil Bhatt, Consultant Radiologist** on 23 Sep 2026

*** End of report ***

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